



1. **What are critical airway red flags?:** AMS with secretions, The inability to speak full sentences, Stridor/silent chest, Recurrent vomiting, Rising CO₂, Increased work of breathing, Declining mental status.
2. **What 3 things should you assess when assessing ventilations?:** Rate, Rhythm, and Quality
3. **What is the purpose of an initial airway assessment?:** To find and treat immediate life threats
4. **What is an example of having a mental model for airway intervention?:** PACE Framework
5. **How do we manage a Foreign Body Occlusion in an UNCONSCIOUS patient?:** If the patient is unconscious, immediately begin CPR., and check the airway after each CPR rotation. Never blind finger sweep
6. **How do we manage a Foreign Body Occlusion in a CONSCIOUS Patient?:** For adults, begin 5 abdominal chest thrusts.
For pediatrics, hold the patient down and provide 5 back blows with the lower palm of your hand, then turn the patient over, and begin 5 downward chest thrusts.
7. **What is complicated about ventilating a patient with an unsupported head/upper back?:** The airway is sitting at an "L" Shape, making ventilations less adequate.
8. **How should we position the airway to provide adequate ventilation?:** Place padding/Support the patients head/upper back to bring the ear to the sternal notch. This creates a much more "straight" path for airflow
9. **How do we apply suction at ATCEMS?:** Using a DuCanto catheter, we suction no more than 10-15 seconds, ventilating between attempts.
10. **What indicates the use of an OPA?:** An unconscious patient without a gag reflex.
11. **What indicates the use of an NPA?:** A semi conscious patient that might have a gag reflex.
12. **What contraindicates an NPA/OPA?:** Facial fractures for NPA, and Mandible/mouth trauma for OPA
13. **What are some S/S of an impending airway failure?:** An increased work of breathing (Retractions, Accessory muscle use), Tripoding, Headbobbing, Change in mental status, Signs of fatigue, Stridor/Gurgling, and Hypoxia/Hypercarbia.
14. **Why don't we use PEEP in cardiac arrest?:** PEEP will increase intrathoracic pressure, putting stress on the vena cava, and increasing preload.



15. **What is the ATCEMS Gold Standard for securing an adequate BVM seal?:** A two handed clamp
16. **What is the IGel?:** A Blind Insertion Airway Device (BIAD) that is sized based off the patients weight. The IGel allows for a suction catheter channel, and isolates the larynx from the pharynx.
17. **What are some pearls of the IGel?:** Using downward pressure to position the device. Always preload the IGel with a G tube, and never use the ET tamer.
18. **What is the PACE plan?:** An airway acronym that plans for a Primary, Alternate, Contingent, and Emergent airway treatment.
19. **What is EtCO₂ and the two components that make up EtCO₂?:** EtCO₂ is the amount of CO₂ that is expired during expiration. It is composed of ventilation and perfusion.
20. **An aerobic metabolism produces ATP efficiently with the Prescence of:** Oxygen
21. **An anaerobic metabolism produces little ATP without oxygen, and allows for:** Lactic Acid buildup
22. **CO₂ will diffuse from the blood into the alveolar space via:** Diffusion, from an area with a low concentration to an area with a high concentration
23. **Anytime an advanced airway is in place, we monitor with an:** Entidal
24. **What can capnography monitor?:** Directly and immediately monitors ventilation. Indirectly monitors perfusion and metabolism. Can show patient trends.
25. **What is the difference between capnometry and capnography?:** Capnometry measures a numerical value, while capnography is able to measure real time via a graph, and numerical values.
26. **What does a bronchospastic waveform look like?:** Sharkfins >^
27. **What are S/S of breathstacking?:** An incomplete exhalation phase, a rising capnography baseline, Increased intrathoracic pressure, Increased risk of barotrauma.
28. **What pneumatic can we use to troubleshoot our airway intervention?:** DOPE
Displacement (Recheck tube placement)
Obstruction (Suction error?)
Pneumothorax (Auscultate lungs)
Equipment (Is my equipment accurately placed/functional?)
29. **What is a normal EtCO₂ value?:** 35-45 mmHg
30. **What is a hypercapnic EtCO₂ reading?:** 45+ mmHg
31. **What is a hypocapnic EtCO₂ reading?:** <35 mmHg
32. **What can a spike in EtCO₂ indicate during CPR?:** ROSC



Airway Management for ATCEMS

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- 33. **Mental status is closely related to...:** Airway Status
- 34. **Upper airway concerns often originate from...:** Patency
- 35. **Lower airway concerns often originate from...:** Ventilation/oxygenation problems
- 36. **What separates the upper and lower airway?:** The larynx